The Logic: Four Levers and One Backbone

Lever	Question it answers	Phases
Affordability	**Can people pay for healthy food?**	**2**
Access	**Can people reach it easily?**	**1, 3**
Ability	**Do people have the time and skills to cook it?**	**4**
Environment	**Is the healthy choice the visible, default one?**	**5**
Accountability (backbone)	**Does the system measure and correct itself?**	**6**

The Six Phases

Phase	Timeline	Core actions	Output

1. Local Diagnosis	**Months 1–3**	**Map food outlets by neighborhood (fresh vs. ultra-processed). Compare price per calorie and per nutrient for a healthy basket vs. an ultra-processed one. Identify areas with the highest diabetes, obesity, and anemia rates. Publish all findings openly.**	**Public report with a neighborhood food-access map**

2. Make Nutritious Food Cheaper	**Months 3–12**	**Redirect subsidies from industrial crops to fruits, vegetables, legumes, and small farmers. Tax sugary drinks and ultra-processed foods, with revenue earmarked for the program. Zero VAT on locally produced fresh produce. Public procurement (schools, hospitals, prisons) prioritizes local producers.**	**A healthy basket with a stable price, competitive with the ultra-processed one**

3. Shorten the Supply Chain	**Months 6–18**	**Neighborhood producer markets at least twice a week. Buying cooperatives for bulk, farm-gate prices. Mobile points of sale for areas with no market. Shared logistics to cut small producers' transport costs.**	**Fresh food within a 10-minute walk in most neighborhoods**
4. Recover Time and Knowledge	**Months 6–24**	**Cooking workshops in schools, community centers, and workplaces (local, quick, low-cost recipes). School meals made from real food bought from local producers. School and community gardens. Recipe books**	**A generation that can cook and values local food**

5. Regulate the Environment	**Months 12–36**	**Mandatory front-of-package warning labels, adapted to each country. Ban ultra-processed food advertising aimed at children under 12. Remove ultra-processed products from vending machines and school shops. Regulate advertising in public spaces and transport.**	**An environment where healthy is visible and easy**
6. Measure and Sustain	**Ongoing**	**Annual indicators (see below). Citizen council of farmers, nutritionists, students, and merchants. Protected budget, independent of the governing**	**A self-correcting system that survives political change**

Indicators (Phase 6 Scorecard)

Dimension	Indicator
Affordability	**Relative price of the healthy basket vs. the ultra-processed basket**
Access	**% of households within 10 minutes' walk of fresh food**
Consumption	**Average fruit and vegetable intake**
Health outcomes	**Overweight prevalence; childhood anemia**
Governance	**Share of the budget protected; frequency of public reports**

Dependencies and Sequencing

1. Phase 1 feeds everything. Its data decides where to prioritize markets, mobile sales points, and workshops.
2. Phases 2 and 3 are linked. Lower prices need shorter chains, and shorter chains need steady demand from public procurement.
3. Phase 2's tax funds the program. Earmarking prevents the money from disappearing into general budgets.
4. Phase 5 comes later because regulation is politically harder once communities already see benefits from Phases 2–4.
5. Phase 6 runs from day one. Baselines come from Phase 1.

Adaptation Checklist

Local variable	What to adjust

Income level	**Tax rates, subsidy size, and the price threshold for “competitive”**
Food culture	**Which foods and recipes count as the healthy basket**
Urban vs. rural	**Fixed markets vs. mobile points and cooperatives**
Institutional capacity	**Which regulations in Phase 5 are realistic and in what**
Main health burden	**Whether to prioritize anemia, obesity, or diabetes in targeting**